the private healthcare industry, and there is market failure in the health insurance industry. [8]

Policy pathways in public health

It is striking to see that there was greater capability in India, in the 1970s, on fighting mosquitoes than is the case today. We have regressed from preventing malaria by fighting mosquitoes in the 1970s, to treating patients who have malaria, dengue or Zika. Problems such as air quality and road safety have become worse in India today than these were many decades ago. There is a need to revive public health, and reduce the extent to which persons require healthcare. Ironically, the weaker the healthcare system, the greater the gains from prevention.

The focus of health policy in India must be upon prevention, i.e., upon public health. This includes immunization, disease vectors such as mosquitoes, monitoring of disease outbreaks and fighting epidemics, drug safety, food safety, air quality, water quality, waste management, disaster risk resilience, disaster response, etc. These areas require prioritization and the development of state capacity.

A lot of public health lies outside the administrative boundaries of the Ministry of Health. As an example, air quality is a key public health crisis that afflicts north India. However, the problem of air quality lies in the Ministry of Environment and not the Ministry of Health. Public health considerations need to shape the working of many ministries. As an example, the Indian road safety crisis is a first-order challenge for public health. The agencies building and operating roads should carry targets for accident rates and not just targets for kilometres of highways built.